

National Clinical Guideline — Heart Failure in Adults

National Health Authority · Clinical Guidance (synthetic)

§1 Classification by ejection fraction

Heart failure is classified by left-ventricular ejection fraction: reduced ejection fraction at or below 40 percent, mildly reduced at 41 to 49 percent, and preserved at or above 50 percent. Diagnosis combines symptoms, signs, natriuretic peptides and echocardiography.

§2 Guideline-directed medical therapy

For heart failure with reduced ejection fraction the four foundational therapies are an ACE inhibitor or ARNI, a beta-blocker, a mineralocorticoid-receptor antagonist, and an SGLT2 inhibitor. SGLT2 inhibitors are recommended regardless of diabetes status and reduce both heart-failure hospitalisation and cardiovascular death.

§3 Diabetes medication considerations in heart failure

SGLT2 inhibitors such as empagliflozin and dapagliflozin are preferred glucose-lowering agents in patients with heart failure. Avoid thiazolidinediones, which cause fluid retention. Metformin is acceptable in stable heart failure but should be withheld during acute decompensation or when eGFR falls below 30.

§4 Monitoring

Review volume status, renal function and potassium within one to two weeks of initiating or uptitrating therapy. Patients with an ejection fraction at or below 35 percent despite optimal therapy should be assessed for device therapy.

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